

# National Clinical Guideline — Cardiovascular Disease Prevention

National Health Authority · Clinical Guidance (synthetic)

## §1 Population risk stratification

Cardiovascular prevention begins with systematic risk estimation using a 10-year ASCVD risk model incorporating age, sex, blood pressure, lipids, smoking and diabetes. Patients are grouped into low, moderate, high and very-high risk to direct the intensity of intervention.

## §2 Statin therapy thresholds

Offer statin therapy to all patients at high or very-high cardiovascular risk, to patients with established atherosclerotic disease for secondary prevention, and to patients with an LDL-C at or above 4.9 mmol/L irrespective of calculated risk. Untreated high-risk patients are a priority care gap.

## §3 Antiplatelet and anticoagulation

Low-dose aspirin is recommended for secondary prevention in established atherosclerotic cardiovascular disease but is not routine for primary prevention. Patients with atrial fibrillation and an elevated stroke risk score should receive a direct oral anticoagulant in preference to warfarin unless contraindicated.

## §4 Lifestyle and risk-factor control

Smoking cessation, the single most cost-effective intervention, should be offered at every contact. Combine blood-pressure control, lipid lowering, glycaemic control and weight management; the absolute benefit is greatest in patients with multiple concurrent risk factors, who are typical of the systemic chronic-disease cohort.

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